

25LBCV03081 25LBCV03081

County: los-angeles

Division: Civil Law and Motion

Department: S28

Hearing date: 2026-09-24

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Case Number: 25LBCV03081 Hearing Date: September 24, 2026 Dept: S28

BACKGROUND

On November 10, 2025, Plaintiff

Mindy Noelle Grasska Friedman ("Plaintiff" or "Friedman") filed a complaint against Andrea Wieck, Jasmyn Ruthie Rachelle Wieck, Jason Wieck, and Does 1 through 10 (collectively, "Defendants") for motor vehicle and general negligence. The action arises out of an alleged automobile collision that took place on November 16, 2023, in Long Beach, California. Plaintiff alleges physical injuries and seeks damages that include medical expenses, wage loss, loss of earning capacity, pain and suffering, and physical limitations affecting aspects of her daily life.

Defendants filed an Answer on February 13, 2026.

On July 6, 2026, Plaintiff filed the instant Motion to Quash and/or Modify Subpoenas and for Protective Order ("Motion"). Defendants filed an Opposition on September 11, 2026, and Plaintiff filed a Reply on September 16, 2026.

DISCUSSION

Applicable

Law

Code

of Civil Procedure section 1987.1, subdivision (a), provides,

(a)

If a subpoena requires the attendance of a witness or the production of books, documents, electronically stored information, or other things before a court, or at the trial of an issue therein, or at the taking of a deposition, the court, upon motion reasonably made by any person described in subdivision (b), or upon the court's own motion after giving counsel notice and an opportunity to be heard, may make an order quashing the subpoena entirely, modifying it, or directing compliance with it upon those terms or conditions as the court shall declare, including protective orders. In addition, the court may make any other order as may be appropriate to protect the person from unreasonable or oppressive demands, including unreasonable violations of the right of privacy of the person.

Analysis

Defendants issued three subpoenas—one for medical records, one for billing records, and one for films or radiology records—to each of the following six providers: Women's Advantage Men's Optimal Health Physical Therapy, MemorialCare Medical Group, Wave Imaging Palo Verde, Constance Shin, LMFT, Mission Advanced Pain Management & Spine Center, and Beverly Radiology Medical Group, for a total of eighteen subpoenas. The subpoenas generally cover the period from November 16, 2013 to the present and seek categories of records pertaining to Plaintiff.

First, Plaintiff requests that the three subpoenas directed to Constance Shin, LMFT, be quashed in their entirety because Plaintiff has not placed her mental health at issue in this litigation, rendering these psychotherapeutic records strictly privileged. Second, Plaintiff requests that the remaining fifteen (15) subpoenas be modified to strike the overbroad language and expressly limit the scope of production to the physical injuries identified in paragraph 6 of the concurrently filed Declaration of Mindy Friedman, which states the physical injuries at issue in this case. Third, Plaintiff asks the Court to order the parties to submit the Stipulation and Protective Order attached as Exhibit 2 to the Declaration of Michael Schachter, based on the Los Angeles Model Protective Order, and that any records produced in response to the subpoenas at issue in this motion be subject to that protective order.

Plaintiff states that on June 16, 2026, and July 5, 2026, she produced all medical records and documents relating to medical expenses from the date of the incident to the present that relate to the specific physical injuries she sustained as a direct result of the subject incident. Plaintiff states that the incident caused serious physical injuries that significantly affect her daily life, including severe neck pain with documented cervical spine conditions, ongoing left-shoulder pain that limits basic physical tasks, debilitating dizziness and headaches that have required multiple medical visits, and persistent fatigue related to her neck injury. Plaintiff does not seek damages for mental health treatment.

Plaintiff's counsel repeatedly sent Defendants a proposed protective order in late May and early June 2026, but Defendants declined to stipulate or narrow their subpoenas. Shortly afterward, on June 12, 2026, Defendants served eighteen subpoenas on six medical and mental-health providers using identical boilerplate language seeking all records from 2013 to the present without any subject-matter limitation.

Plaintiff states that her medical records, including documents related to her accident injuries, contain highly private information about unrelated conditions, including sensitive pregnancy-related details and family medical history. Such unrelated information routinely appears within records addressing her incident-related injuries, which is why she redacted non-incident-related material before producing her records.

In their Opposition, Defendants argue that the subpoenas are necessary because Plaintiff's own production was incomplete, filtered, and withheld key information. Plaintiff's ongoing treatment meant her production could not be comprehensive, later records were added, and her claims summary removed items she deemed unrelated. Her interrogatory responses also reserved or withheld substantive details about injuries, symptoms, treatment, prior complaints, and economic losses. Defendants contend provider-direct subpoenas are needed to verify completeness, obtain authenticated records, review the full treatment history, evaluate medical findings, confirm billing information, and obtain full imaging.

Moreover,

Defendants argue that Plaintiff's prelitigation submissions to Defendants' insurer are irrelevant to the present discovery dispute as these submissions are not a substitute for formal discovery and may be incomplete and unauthenticated. Nor does an insurer's fault determination control Defendants' litigation rights or limit their ability to conduct discovery.

Defendants

also argue that the subpoenas cover a defined ten-year period before the incident and seek records regarding the onset, cause, or aggravation of the injuries at issue, especially given Plaintiff's degenerative cervical and thoracic findings and other potential contributing factors reflected in her records. Pre-incident records may be relevant because Plaintiff refused to say whether prior similar complaints existed and declined to produce any such records. If overbreadth is a concern, the proper remedy is narrowing the subpoenas, not quashing them entirely.

Finally,

Defendants maintain that any privacy concerns can be addressed with "reasonable subject-matter limitations" or "targeted redactions" as they do not seek public dissemination of Plaintiff's medical information. (Opposition, pp. 4-5).

In

her Reply, Plaintiff argues that Defendants concede many of Plaintiff's arguments. They agree that the subpoenas should be modified rather than enforced as written, acknowledge that the ten-year timeframe can be reasonably narrowed, and confirm that ordinary pain and suffering does not waive psychotherapist privilege. They also state that privacy concerns can be addressed through subject-matter limits, redactions, or in camera review. These concessions align with Plaintiff's request to modify the fifteen subpoenas. Moreover, despite claiming they do not seek public disclosure of Plaintiff's medical information, Defendants filed Plaintiff's medical records on the public docket without redaction or a sealing request, underscoring the need for the protective order they previously refused. Plaintiff is not seeking to bar discovery from her providers but rather opposing Defendants' attempt to obtain her entire medical history. With a protective order in place, provider-direct production would give Defendants the complete, unredacted records they claim to need. Defendants also omit that they deposed Plaintiff on August 19, 2026, questioning her extensively about her injuries, treatment, imaging, surgical recommendations, medications, and functional

limitations.

A. Subpoenas

Directed to Constance Shin, LMFT

The

three subpoenas directed to Constance Shin, LMFT seek Plaintiff's mental health and psychotherapeutic records.

(Schachter Decl., Exs. 12-14.)

Under

Evidence Code section 1014, confidential communications between a patient and a psychotherapist are absolutely privileged.

However, the privilege is waived "concerning the mental or emotional condition of the patient if such issue has been tendered by" the patient. (Evid. Code, §1016.) Here, Plaintiff does not seek mental health

treatment damages and expressly states in her declaration that she is not seeking damages for mental health treatment in this case. The billing subpoena to Shin also seeks diagnostic codes, procedure codes, and dates/frequency of psychotherapy, but

Defendants offer no showing of relevance connecting dates or frequency of counseling to Plaintiff's physical injuries.

Moreover, diagnoses are encompassed in the definition of "confidential communication between patient and psychotherapist." (Evid. Code, § 1012.) Thus, section 1016 does not apply and the privilege is not waived.

Defendants

argue that Plaintiff should clarify whether she intends to claim any psychological or emotional impairment. If she does, limited discovery into that condition would be appropriate; if she does not, her psychotherapy communications should remain protected, with only relevant non-privileged administrative information discoverable. They also ask that Plaintiff not be permitted to later add mental-health damages without reopening related discovery.

The

Court finds the subpoenas improper. Plaintiff's mental health is not at issue and all of the psychotherapy communications, including diagnostic information, remain privileged. The Motion to Quash the three subpoenas directed to Constance Shin, LMFT is GRANTED.

The Court does not grant Defendants' request, made in the Opposition, for an order prospectively precluding Plaintiff from altering her damages

claims as Defendants provide no basis or authority for this request.

B. Subpoenas

to Physical Medical Providers

The fifteen subpoenas at issue seek

all medical, billing, and radiology records without subject-matter limitation. According to Code of Civil Procedure section 2020.410, subdivision (a), “[a] deposition subpoena that commands only the production of business records for copying shall designate the business records to be produced either by specifically describing each individual item or by reasonably particularizing each category of item.” A blanket demand for documents may properly be quashed when its breadth makes it a fishing expedition. (*People v. Serrata* (1976) 62 Cal.App.3d 9, 15 (1976).) Here, the subpoenas are overbroad.

Moreover, California’s Constitution

expressly grants a right to privacy that includes the protection of medical information. (*Williams v. Superior*

Court (2017) 3 Cal.5th 531, 552; *Hill v. National Collegiate Athletic*

Assn. (1994) 7 Cal. 4th 1, 35 (“Legally recognized privacy interests...[include] interests in precluding the dissemination or misuse of sensitive and confidential information.”) “A person’s

medical profile is an area of privacy infinitely more intimate, more personal in quality and nature than many areas already judicially recognized and protected.” (*Board of Medical Quality*

Assurance v. Gherardini (1979) 93 Cal.App.3d 669, 678, overruled in part on other grounds.)

Plaintiff has shown that her medical

records— even those relating to claimed incident-related injuries—contain unrelated private information, such as sensitive pregnancy-related details and family medical history. Defendants declined Plaintiff’s early request for a protective order governing these records.

Defendants

contend that any modification should allow discovery of records relating to all conditions, diagnoses, symptoms, and contributing factors reflected in Plaintiff’s records, while still excluding unrelated medical information.

In

her Reply, Plaintiff proposes the following modification to the subpoenas:

Production shall be limited to records concerning the diagnosis, treatment, or evaluation of the following conditions and anatomical regions: the cervical spine, thoracic spine, neck, left shoulder, and left upper extremity, including cervicalgia, cervical spondylosis, central canal and foraminal stenosis, disc protrusion and disc bulge, cervical and thoracic radiculopathy, and associated neurologic symptoms; and dizziness, headaches, and fatigue.

As to those conditions and regions only, production shall include the related examination and progress notes, diagnostic imaging and reports, medications, treatment rendered, prognosis, surgical recommendations, billing and payment records, and any documented physical restrictions arising from those conditions. This limitation does not extend to records concerning obstetric, gynecologic, prenatal, postpartum, reproductive, hormonal, or mental health care, or to any condition not listed above.

(Reply,
p. 6.)

The Court finds that the proper remedy is to modify the subpoenas. Here, Plaintiff has established a legally protected interest and Defendant has raised an important countervailing interest in obtaining the information for purposes of litigation. (See Williams, 3 Cal. 5th at 552.) The Court must balance these competing considerations. (Ibid.) The Court grants Plaintiff's request to impose subject matter limitations on the subpoenas and rejects an unrestricted category for alternative or contributing causes proposed by Defendants as it is not tied to a claimed condition or anatomical region and would reintroduce the same catch-all defect that requires modification. The Court also declines to authorize discovery into all medical conditions that might affect parenting, childcare, employment, or activities of daily living.

Accordingly, the fifteen subpoenas shall be modified to limit the subject matter at issue and preclude production of records concerning obstetric, gynecologic, prenatal, postpartum, reproductive, hormonal, or mental

health care. The Court declines to include an untethered category for “alternative or contributing causes” as an independent subject; causation and aggravation discovery is preserved as limited to the defined anatomical regions and conditions above, which is sufficient for relevance and proportionality.

C. Protective Order

Plaintiff

argues that even when properly narrowed, her medical records will inevitably contain highly sensitive, unrelated medical information related to her pregnancy and family medical history.

Although Plaintiff’s counsel has attempted to obtain Defendants’ stipulation to a standard Los Angeles Model Protective Order, Defendants have refused. Plaintiff requests that the Court order the parties to submit the Stipulation and Protective Order, attached as Exhibit 2 to Michael Schachter’s declaration, and order that any records produced in response to the subpoenas at issue in this Motion be subject to that protective order.

Defendants

argue Plaintiff has not shown good cause for a protective order. They note that a court must balance interests before restricting dissemination and that the party seeking protection bears the burden of demonstrating specific harm. Plaintiff’s assertion that her records contain “highly private, sensitive information” about unrelated conditions is viewed as conclusory, lacking detail about any actual risk of annoyance, embarrassment, or oppression. Defendants therefore contend these statements do not justify a blanket protective order.

As

noted above, Defendants publicly filed Plaintiff’s medical records—Exhibits E through H—without redaction, sealing, or in-camera review. These records revealed diagnoses, potential surgical plans, reproductive-health information unrelated to the case, and notes about Plaintiff’s mental state. Because Defendants placed this sensitive information on the public docket while the motion was pending, the risk of further disclosure will continue without a protective order.

The

Court finds that Plaintiff has shown that records addressing the physical injuries at issue may include sensitive information about unrelated medical conditions, pregnancy, family members, procedures, and medications. Pursuant to Code of Civil Procedure section 1987.1, the Court may condition subpoena compliance on a protective order and may take measures necessary to prevent unreasonable invasions of privacy.

The

Court GRANTS Plaintiff's request for a protective order.

CONCLUSION

Plaintiff

Mindy Noelle Grasska Friedman's Motion to Quash and/or Modify Subpoenas and for Protective Order is GRANTED IN PART as follows:

The

medical-records, billing-records, and films or radiology subpoenas directed to Constance Shin, LMFT are QUASHED in their entirety.

The

15 subpoenas directed to Women's Advantage Men's Optimal Health Physical Therapy, MemorialCare Medical Group, Wave Imaging Palo Verde, Mission Advanced Pain Management & Spine Center, and Beverly Radiology Medical Group are MODIFIED as follows:

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Production shall be limited to records

concerning the diagnosis, treatment, evaluation, or documented history of the following conditions and anatomical regions: the cervical spine, thoracic spine, neck, left shoulder, and left upper extremity, including cervicgia, cervical spondylosis, central-canal or foraminal stenosis, disc protrusion or disc bulge, cervical or thoracic radiculopathy, and associated neurologic symptoms; and dizziness, headaches, and fatigue. As to those conditions and anatomical regions only, production may include examination and progress notes, prior similar complaints, diagnostic imaging and reports, medications, treatment rendered, prognosis, surgical recommendations, billing and payment records, and documented physical restrictions arising from those conditions. Records concerning a possible alternative or contributing cause may be produced only to the extent the record expressly connects that cause to one or more of the conditions or anatomical regions identified above.

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Excluded

matters. Production

shall not include records concerning unrelated obstetric, gynecologic, prenatal, postpartum, reproductive, hormonal, or mental-health care, except that a discrete portion of an otherwise responsive record may be produced if it expressly connects such a matter to a condition or anatomical region identified in the previous paragraph. Any

production under this exception shall be narrowly limited to the expressly stated connection and shall otherwise be redacted.

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The

subpoenas remain limited to records dated from November 16, 2013 through the date of production.

Within 10 days of the Court's order,

the parties must submit a stipulated protective order substantially in the form of Exhibit 2 to the Schachter Declaration, with any mutually agreed revisions, for signature. All

records produced in response to the modified subpoenas shall be designated and treated as Confidential under that order unless otherwise agreed or ordered. No party shall publicly file medical records produced pursuant to these subpoenas without redacting information excluded by this Order or obtaining an appropriate court order governing the filing.